Trastuzumab subcutaneous (SC) · early breast cancer

Trastuzumab 600 mg under the skin every 21 days, without a loading dose, to complete one year total. Previous intravenous (IV) or combination-treatment trastuzumab counts towards that year; a route switch does not start a new year.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Breathing

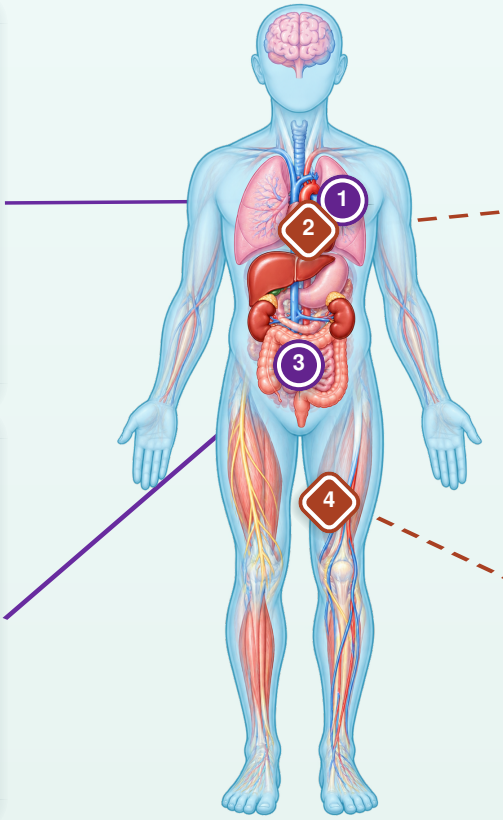
Trastuzumab

New cough or breathlessness: seek prompt assessment.

2 Heart

Trastuzumab

Keep echo checks. Report swelling, chest symptoms, fainting or rapid weight gain.



3 Bowel / fatigue

Trastuzumab

Report persistent diarrhoea, dehydration or fatigue affecting daily activity.

4 Injection site

SC trastuzumab

Report painful, spreading redness or swelling where the injection was given.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

> Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

Your treatment, at home.

Important medicine-specific effects

Trastuzumab

- Heart pumping-function reduction / heart failure
- Allergic or administration-related reactions
- Injection-site pain, redness or swelling
- Diarrhoea, fatigue and rare serious lung effects

Surgery, radiation and monitoring: open the whole-treatment map or scan the QR.

Make the plan practical

Support for this medicine

Trastuzumab alone has minimal emetogenic risk. Routine chemotherapy antiemetics, steroids and growth factors are not added automatically. Previous reactions require a separate plan.

Blood and heart appointments

full blood count (FBC), renal and liver profile every 6 weeks (around C3, C5, C7 if on time). Echo / multigated acquisition (heart pumping) scan (MUGA) every 12 weeks; around C5 for uninterrupted 21-day cycles.

Local treatment and endocrine therapy

Surgery and radiotherapy are coordinated by the multidisciplinary team (MDT). When endocrine therapy is indicated, it has its own schedule and bone-health plan.

Pregnancy / contraception

Discuss pregnancy, fertility and contraception before treatment. Report pregnancy promptly; do not breastfeed during treatment or for 7 months afterwards. The team confirms the medicine-specific contraception duration.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Complete reference, visual timeline and passport

https://sactcheck.com/patient/00285/

Sources and review status are available online. Follow your own prescription and hospital instructions.